DPP	DEPARTMENT POLICY AND PROCEDURE		
Title	**TRACHEOSTOMY SUCTIONING AND CARE**		
Department	**Pediatric Intensive Care Unit**		
Applies to	**All PICU Medical and Nursing Staff**		
Policy Code and Version Number	**DPP-PICU-019 (v2)**		
Last Version Approval Date	**15/12/2022**	Number of Pages	5

	NAME	POSITION	SIGNATURE	DATE
Prepared by:	**MS. REMYA MOHANDAS**	Quality Representative, Pediatric Intensive Care Unit		23/11/25
	MS. SARA ZAIBI AL SHOMRANI	Head Nurse, Pediatric Intensive Care Unit		23-11-25
	DR. LOTFI DJAIDER	Pediatric Consultant		23-11-25
Reviewed by:	**DRA. FAWZIA MOHD FRAHAT**	Head of Dept /Consultant , Pediatric Intensive Care Unit		7-12-25
	MR. MANSOUR AHMED AL-ZAHRANI	Head, Quality Management and Patient Safety		7-12-25
Approved by:	**MR. ABDULMAJEED MOHD AL-SOHEIMI**	Nursing Director		11-12-25
	DR. YOUCEF SOUAYEH	HED Assistant for Medical Affairs		11.12.25

APPROVAL DATE	EFFECTIVE DATE	TYPE OF REVISION	REVIEW DATE
11 / 12 / 2025	25/ 12/ 2025	TRIENNIAL	10 / 12/ 2028

Baha_Cluster
Healthcluster-baha@moh.gov.sa
مستشفى المخواة العام
Al-Mikhwah General Hospital

1.0 PURPOSE

1.1. To prevent infection and to maintain mucus membrane and skin integrity.

1.2. To prevent accumulation of secretions that can interfere with respiration thus improving ventilation and gas exchange.

1.3. To ensure airway patency by keeping the tube free of mucus build up.

2.0 DEFINITION/ABBREVIATION

2.1. **Tracheostomy care** is the care rendered to patient with an artificial hole through the neck to the windpipe. The airway is kept open, humidity is provided and the wound is kept sterile. Without such care patient could suffer injury to the vocal cords, stomach problems, blockage of the windpipe and infection.

2.2. **H202** - Hydrogen Peroxide

2.3. **BVM**- Bag Valve Mask

3.0 RESPONSIBILITY

3.1 **Head Nurse** is responsible to evaluate staff nurses about their knowledge in caring patient with tracheostomy and to teach them as well if indicated.

3.2 **Charge Nurses** are responsible to make sure that patient's safety is ensured during the procedure.

3.3 **Staff nurses** are responsible to perform tracheostomy care, assess and re-assess patient in a timely manner and to report any signs of infection in the stoma.

3.4 **Respiratory therapist** is responsible in the maintenance of ventilator or other oxygen-delivering devices and may assist in suctioning of secretions.

4.0 POLICY

4.1. Tracheostomy care should be performed using aseptic technique.

4.2. Gloves must be used for all manipulation at the tracheostomy site.

4.3. The nurse should focus on maintaining patency of airway, facilitating the removal of pulmonary secretions and cleansing the stoma.

4.4. Document proper tube placement.

4.5. The staff must closely monitor signs of complication such as hemorrhage, edema around the stoma, accidental de-cannulation free air into the pleural cavity.

4.6. The physician should be notified for any complications.

4.7. The staff can prevent complications by assessing the patient during patient interaction and tracheostomy care.

Baha_Cluster
Healthcluster-baha@moh.gov.sa
مستشفى المخواة العام
Al-Mikhwah General Hospital

5.0 PROCEDURE (S)

5.1. ***Preparatory phase***

5.1.1. Wash hand thoroughly.

5.1.2. Assemble equipments and supplies. Check expiration date on sterile package and inspect for tears.

5.1.3. Assess the patient's condition and condition of stoma such as redness, swelling, character of secretions presence of purulence or bleeding.

5.1.4. Explain the procedure to the patient. Provide privacy.

5.1.5. Place the patient in semi-fowler's position unless it is contraindicated.

5.2. ***Performance phase***

Sterile Tracheobronchial Suctioning

5.2.1. Check function of suction and BVM connected to 100% oxygen source. Open catheter suction package. Open sterile towel and place on patient's chest.

5.2.2. Put on face shield and sterile gloves. Designate one hand as contaminated for disconnecting, bagging, and working the suction control. Usually the dominant hand is kept sterile and will be used to thread the suction catheter.

5.2.3. Use the sterile hand to carefully remove the suction catheter from the package, curling the catheter around the gloved fingers.

5.2.4. Connect suction catheter to suction source with the contaminated hand,

5.2.5. Using contaminated hand, disconnect the patient from the ventilator, CPAP device or other oxygen source. (Place ventilator connector on the sterile towel)

5.2.6. Ventilate and oxygenate the patient with the BVM for four to five times, If possible with another person (a nurse or respiratory therapist)

5.2.7. Gently insert suction catheter as far as possible into the artificial airway without applying suction. Most patients cough when catheter touches carina.

5.2.8. Withdraw the catheter % inch-l inch (2-3cm) and apply suction. Quickly rotate catheter while it is being withdrawn.

5.2.9. Limit the suction to not more than ten seconds

5.2.10. Bag patient between suction passes with approximately four to five manual ventilations. must be replenished before suctioning is attempted again.

5.2.11. Rinse catheter in between passes. To remove thick secretions from catheter.

5.2.12. Continue making suction passes, bagging the patient between passes, until the airways are clear of accumulated secretions.

5.2.13. Return the patient to the ventilator or apply CPAP or other oxygen- delivery device

Tracheostomy Care

5.2.14. Remove soiled tracheostomy dressing. Reduces microorganisms.

5.2.15. Remove soiled gloves and prepare prepacked kit or supplies; prepare supplies and place approximately 100 in two bowls, one with hydrogen peroxide and the other with normal saline.

5.2.16. Don new procedure gloves, Remove tracheostomy collar and oxygen supply.

For Disposable Inner Cannula

5.2.17. Clear the external end of the tracheostomy tube with 2 gauze sponges with hydrogen peroxide.

5.2.18. Clear the stoma area with 2 peroxide-soaked gauze sponges. Make only a single sweep with each gauze sponge before discarding.

5.2.19. Loosen and remove crust with sterile cotton swabs then clear the stoma area with sterile water-soaked gauze sponges.

5.2.20. Clean the stoma area using dry sponges. Ensures dryness of the area.

5.2.21. Change a disposable inner cannula, touching only the external position and lock it securely into place.

For Reusable Inner Cannula

5.2.22. Remove inner cannula and place into H202 bowl.

5.2.23. Cleanse inner cannula with small brush or pipe stem cleaners.

5.2.24. Rinse inner cannula with Normal Saline poured over the cannula and let soak; remove and let cannula dry

5.2.25. Clear the stoma area with 2 peroxide-soaked gauze sponges. Make only a single sweep with each gauze sponge before discarding.

5.2.26. Loosen and remove crust with sterile cotton swabs then clear the stoma area with sterile water-soaked gauze sponges.

5.2.27. Insert and lock inner cannula in place. Secures inner cannula.

5.2.28. Change tracheostomy tie tapes. Cut soiled tape while holding tube securely with other hand. careful not to cut the pilot balloon tubing.

Baha_Cluster
Healthcluster-baha@moh.gov.sa
مستشفى المخواة العام
Al-Mikhwah General Hospital

5.2.29. Remove old tapes carefully. Grasp slit end of clear tape and pull it through opening on side of tracheostomy tube. Pull other end of tape securely through the slit end of the tape. Repeat on the other side then tie the tapes at the end of the neck in a square knot. Alternate knot from side to side each time tapes are changed.

5.2.30. Ties should be tight enough to keep tube securely in the stoma, but loose enough to permit two fingers to fit between the tapes and the neck.

5.2.31. Place a gauze pad between the stoma site and the tracheostomy tube. Always keep the area dry.

5.2.32. Cleaning of the fresh stoma should be performed every 8 hours or more frequently if indicated by accumulation of secretions. Ties should be changed frequently if soiled or wet.

5.3. ***Follow up phase***

5.3.1. Document procedure performance, observations of stoma and character of secretions. Report changes in stoma appearance or secretions.

6.0 EQUIPMENTS

6.1. None

7.0 ATTACHMENT/FORMS

7.1. N/A

8.0 REFERENCES

8.1. MOH Guidelines for Pediatric Care

8.2. DPP-PICU-0024(V2) –Care of Patient with Endotracheal Tube– PMSH, 15/2/1439

8.3. Saudi Central Board for Accreditation of Healthcare Institution (CBAHI) PICU standards, 3rd version, January 2016

9.0 REVISION CLAUSE

9.1. Used the new logo for AL-Baha Health Cluster.

9.2. Modify the policy and procedure lay out, changed the content of the Header and footer

Baha_Cluster
Healthcluster-baha@moh.gov.sa
مستشفى المخواة العام
Al-Mikhwah General Hospital